Document Control Report

Title Transfer of Adult Critical Care Patients Policy			
Authors Joerg Kuehne Donna Knight Alison Smith		**Author's job title** Clinical Lead Anaesthetics/Theatres and Critical Care Senior Nurse Matron Critical Care Sister Critical Care	
Directorate Anaesthetics, Theatres and Critical Care	**Sub-directorate**	**Department** Intensive Care and High Dependency	**Team / Specialty** Critical Care

Version	Date Issued	Status	Comment / Changes / Approval
0.1	Dec 2011	Draft	Initial version for consultation.
0.2	Jan 2012	Draft	Feedback from consultation
0.3	Jan 2012	Draft	Feedback from Senior Medical and Nursing Team
0.4	Oct 2012	Draft	Amendments by Corporate Governance to document control report, version control, formatting for document map navigation and table of contents.
0.5	Oct 2012	Draft	Amendments from Assistant Director of Nursing and feedback from Corporate Governance.
0.6	Feb 2013	Draft	Further amendments and addition of transfer form (Appendix B).
0.7	Dec 2013	Draft	Further amendments following review at Patient Safety Operational Group
1.0	June 2014	Final	Approved by Patient Safety Operational Group in May 2014 and published on Bob

Main Contact Donna Knight ICU/HDU Level 3 North Devon District Hospital Raleigh Park Barnstaple, EX31 4JB **Lead Director** Director of Nursing	Tel : Direct Dial – 01271 322708 Tel : Internal – x2708 Fax : Email : donnaknight1@nhs.net
Document Class Policy	**Target Audience** All Trust and Sodexho staff involved in the escort and transfer of patients between wards, departments, hospitals internal to, and external to the Trust.
Distribution List Senior Management	**Distribution Method** Intranet site

Superseded Documents
NDHT Transfers from or to Critical Care Guidance
ITU Inter-hospital Transfer Procedural Guide, Dr Sowden, Dr Kuehne June 2007.

Issue Date	Review Date	Review Cycle
February 2013	February 2016	Three years

Consulted with the following stakeholders: (list all) • Consultant and senior nursing staff Critical Care, Accident and Emergency, Resuscitation teams, theatres and Recovery. • Senior Nurses and Ward Managers • Hotel Services Coordinator • Compliance Manager	**Contact responsible for implementation and monitoring compliance:** Senior Nurse/Matron Critical Care- Formal Audit of inter hospital transfers undertaken by Torbay Hospital. **Education/ training will be provided by:** • Clinical Educator and Senior Nursing team Critical Care.
Approval and Review Process • Patient Safety Operational Group	**Ratified by Trust Board?** No
Local Archive Reference G:\CORPORATE NURSING **Local Path** Policies\Escort and Transfer of Patients **Filename** Transfer of adult critical care patients policy V1.0 04Jun14 (Right click text above to update to new filename, same with footer).	
Policy categories for Trust's internal website Nursing	**Tags for Trust's internal website**

Contents

1 Introduction 4
2 Purpose 4
3 Definitions 4
3.1 Critically ill patient 4
3.2 Intra-hospital patient transfer 5
3.3 Inter-hospital patient transfer 5
3.4 Transfer team 5
3.5 Patient Groups 5
4 Responsibilities 5
4.1 Role of Director of Nursing 5
4.2 Role of Duty Critical Care Consultant 6
4.3 Role of Critical Care Doctor 6
4.4 Role of Critical Care Co-ordinator 7
4.5 Role of the Critical Care Registered Nurse 7
4.6 Role of Emergency Department and Ward Registered Nurses 8
4.7 Role of Support Staff and Porter 8
4.8 Role of the Student Nurse 8
5 Transfer Procedure 9
5.1 Decision to transfer 9
5.2 Prior to Transfer 9
5.3 During Transfer 10
5.4 Repatriations 11
6 Training requirements 11
7 Monitoring Compliance With and the Effectiveness of the Policy 11
7.1 Standards/ Key Performance Indicators 11
7.2 Process for Monitoring Compliance and Effectiveness 11
8 References 12
9 Associated Documentation 12
Appendix A: Transfer flowchart 14
Appendix B: Critical Care Transfer Form 15

1 Introduction

This document sets out Northern Devon Healthcare NHS Trust's system for the emergency or planned escort and transfer of adult critical care patients between departments and other hospitals (both internal and external to the Trust). It provides a robust framework to ensure a consistent approach across the whole organisation, and supports our statutory duties as set out in the NHS Constitution.

This policy is intended for all critically ill adult patients transferred at any time of the day or night, and is applicable on all days of the week.

For the transfer of non critically ill patients please refer to the Transfer and Escort of Adult Patients Policy.

2 Purpose

The purpose of this document is to ensure that all critically ill patients will be transferred safely between departments and hospitals, and other healthcare settings, both internal and external to the Trust, without compromising their condition whilst maintaining high standards of care. This policy has been developed in response to the recognised risk to patients, personnel and equipment used when transferring critically ill patients between safe environments and follows Intensive Care Society guidelines and recommendations where possible.

Implementation of this policy will ensure that:

- The risks associated with transferring patients are reduced.
- The appropriate personnel and equipment are involved in the transfer.
- Patients are transferred safely and without compromise to their condition.
- Complete and accurate communication occurs between the sending and the receiving critical care areas, department or hospital.
- The risk of transmitting community or hospital-acquired infections is reduced.
- At all times, consideration is given to the preservation of a patient's dignity and to their psychological and physical needs.

This policy excludes those patients retrieved by specialist centres i.e. paediatric and patients requiring specialist ventilation i.e. ECMO, HFOV.

The policy applies to all Trust staff and staff employed by Sodexo Healthcare who are involved in the escort and transfer of patients between wards, departments, hospitals internal to, and external to the Trust.

3 Definitions

3.1 Critically ill patient

Critically ill patients in this policy are defined as requiring a level of care greater than that normally provided on a standard ward.

3.1.1 Levels of Care

Level 0: Requires hospitalisation needs can be met through normal ward care.

Level 1: Recently discharged from a higher level of care, in need of additional monitoring/clinical interventions, clinical input or advice.

Level 2: Patients needing pre-operative optimisation extended post operative care, stepping down from Level 3.

Level 3: Patients receiving advanced respiratory support alone, advanced cardiovascular support alone or a minimum of 2 organs supported.

3.2 Intra-hospital patient transfer

Critically ill adult patients moved internally within the hospital in which they are currently an inpatient or under assessment.

3.3 Inter-hospital patient transfer

Critically ill adult patients moved from one hospital, or healthcare setting, to another. This may be a transfer from one critical care setting to a specialist setting or similar provider. External transfers relate to those that are planned and emergency which maybe initiated due to clinical need or in times when demand exceeds capacity within Trust.

3.4 Transfer team

The transfer team can consist of one or more members of the following doctors, nursing, operating department assistants, and paramedics etc who are suitably trained and competent to accompany critically ill patients to areas outside of the immediate environment, both internal and external to the Trust.

3.5 Patient Groups

Patients in the following groups should be transferred in accordance with this Policy:

3.5.1 Inpatients

A patient who is admitted to a critical care environment i.e. Intensive Care/High Dependency Unit, or Cardiology ward who requires ongoing specialist treatment and interventions which are not provided within this Trust.

3.5.2 Patients undergoing emergency assessment in ED

A critically ill patient who has been assessed to require ongoing critical care treatment and management,either within the Trust or transfer to a specialist centre.

There should be no differentiation on the quality and safety of transferring these different groups of patients.

4 Responsibilities

4.1 Role of Director of Nursing

The Director of Nursing is responsible for ensuring there are robust processes in place for the safe escorts and transfer of critically ill adult patients, and ensuring that all senior management are aware of this policy.

4.2 Role of Duty Critical Care Consultant

The Duty Intensive Care Consultant is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines for "The transport of the critically ill adult (3rd Edition 2011)" are followed in order to promote high standards of care during transfer.
- The clinical care of the critically ill patient in collaboration with the patients admitting consultant.
- The decision to transfer a patient to another hospital in conjunction with their consultant colleague from the relevant speciality in the receiving hospital.
- Deciding on the most appropriate mode of transport to be used in conjunction with South West Ambulance Service Trust, i.e. road via ambulance, air by helicopter (Devon Air Ambulance daytime only, or RAF Sea King day and night).
- Assessing the requirement of the patient and allocating a suitably trained and competent member of staff when planning the transfer of a patient outside of the critical care environment.
- Ensure the escalation plan is initiated in accordance with Trust and national policy via Bleep 500 where non clinical transfers for capacity reasons are required (ICS Recommendation 7.4.3).

4.3 Role of Critical Care Doctor

The doctor is responsible for ensuring:

- They are suitably trained and competent to undertake the transfer.
- They have full, accurate, and up to date knowledge of the patient in their care in order to provide an appropriate and safe handover to the receiving healthcare setting.
- Ensuring that the patient is wearing a patient identification name band in accordance with the Patient Identification Policy.
- The patient and their next of kin (NOK) / carer have been informed of the need to transfer. The information given to the patient will be dependant on their condition and level of consciousness. Where possible NOK or relatives/carers will be given contact details of the receiving hospital/unit and patients property.
- Completed and photocopied appropriate paperwork for transfer i.e. ED assessment information, Admission and Nursing Assessment, Risk Assessment Booklet, Care Plan and any other relevant paperwork.
- Ensure all Personal Protective Equipment is worn as appropriate i.e. reflective jacket, fire retardant flying suit, boots, warm fleece, ear defenders, helmet and universal infection control measures are adopted as appropriate according to Standard Infection Control Precautions Policy.
- Ensure the critically ill patients management and support is monitored throughout and observations documented on the Critical Care Transfer Form. This includes responding to any change in the patient's condition.
- Ensuring that a patient's dignity, nutritional and safety needs are met during the period of transfer.
- Ensure a comprehensive handover is given to the receiving hospital.

4.4 Role of Critical Care Co-ordinator

The nurse co-ordinator in charge is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines for The transport of the critically ill adult (3rd Edition 2011) are followed in order to promote high standards of care during transfer.
- Undertake risk assessment and collaborate with the Duty Intensive Care Consultant where, when and how the transfer will take place.
- Assess the requirement of the patient and allocate a suitably trained and competent member of staff when planning the transfer of a patient outside of the critical care environment.
- Ensure all patient transfer equipment and personal protective equipment is checked and fit for purpose prior to transfer.
- Ensuring the transfer is undertaken safely and timely in response patient's clinical needs.
- Ensure all transfer personnel are aware of any risks related to the patients transfer i.e. deterioration, resuscitation status, infections, pressure ulceration etc.
- Liaise with Bleep 500 and Patient Transport (in hours) for authorisation to arrange a taxi from the receiving hospital to return transferring personnel and equipment.

4.5 Role of the Critical Care Registered Nurse

The registered nurse is responsible for ensuring:

- They are suitably trained and competent to undertake the transfer.
- They have full, accurate, and up to date knowledge of the patient in their care in order to provide an appropriate and safe handover to the receiving healthcare setting.
- Ensuring that the patient is wearing a patient identification name band in accordance with the Patient Identification Policy.
- The patient and their next of kin / carer have been informed of the need to transfer. The information given to the patient will be dependant on their condition and level of consciousness. Where possible next of kin or relatives/carers will be given contact details of the receiving hospital/unit and patients property as per Trust policy.
- Complete and photocopy appropriate paperwork prior to transfer i.e. ED assessment information, Admission and Nursing Assessment, Risk Assessment Booklet, Care Plan and any other relevant paperwork.
- Ensure all Personal Protective Equipment is worn as appropriate i.e. reflective jacket, fire retardant flying suit, boots, warm fleece, ear defenders, helmet and universal infection control measures are adopted as appropriate according to Standard Infection Control Precautions Policy.
- Ensure the critically ill patients management and support is monitored throughout and observations documented on the network transfer form. This includes responding to any change in the patients condition

- Ensuring that a patient's dignity, nutritional and safety needs are met during the period of transfer.
- Ensure a comprehensive handover and appropriate documentation prior to and during transfer is completed and given to the receiving hospital on arrival.

4.6 Role of Emergency Department and Ward Registered Nurses

The Registered nurse is responsible for ensuring:

- They are suitably trained and competent to undertake the transfer.
- They have full, accurate, and up to date knowledge of the patient in their care in order to provide an appropriate and safe handover to the receiving healthcare setting.
- Ensuring that the patient is wearing a patient identification name band in accordance with the Patient Identification Policy.
- The patient and their next of kin / carer have been informed of the need to transfer. The information given to the patient will be dependant on their condition and level of consciousness. Where possible Next of Kin or relatives/carers will be given contact details of the receiving hospital/unit and patients property as per Trust policy.
- Complete and photocopy appropriate paperwork prior to transfer i.e. ED assessment information, Admission and Nursing Assessment, Risk Assessment Booklet, Care Plan and any other relevant paperwork.
- Ensure the critically ill patient is monitored throughout and observations including EWS according to Management of Patient at Risk Policy are documented on the patients observation chart. The nurse must be able to respond to any change in the patients condition.
- Ensuring that a patient's dignity and safety needs are met during the transfer.
- Ensure a comprehensive handover and appropriate documentation prior to and during transfer is completed and given to the receiving hospital on arrival.

4.7 Role of Support Staff and Porter

The support staff and porter are responsible for:

- Following any instructions given by the nurse in charge of the patient in relation to the transfer of the patient.
- Responding to any request for assistance in the transfer of patients within the hospital.

4.8 Role of the Student Nurse

Student nurses should only undertake internal and external transfers following a risk assessment and must be under continuous supervision by a competent critical care nurse.

5 Transfer Procedure

5.1 Decision to transfer

Patients should be appropriately resuscitated and stabilised prior to transfer to reduce the physiological disturbance related to movement and reduce the risk of deterioration during transfer, as recommended by Intensive Care Society Standards and Guidelines 17.12.1.

The decision to transfer a patient is based on clinical need and safety. Where possible the ICS recommends critically ill patients should not be transferred between 22:00hrs and 07:00hrs. In extreme situations this may be unavoidable due to clinical need and every measure must be taken to reduce risk.

The requirement to transfer a patient to and from a critical care area must be assessed by the Doctor in charge of the patient, unit/ward co-ordinator and the nurse looking after the patient.

5.2 Prior to Transfer

This procedure is applicable to all critically ill adult patient transfers. Prior to transfer, the registered nurse will:

For Transfer Flow chart see Appendix A.

- Explain the transfer procedure to the patient.
- Ensure that the patient is aware of the reason for transfer where possible. Next of kin or significant others who may be affected by the patient's transfer should be given relevant details as soon as is practicable
- Verify the patient's identity in accordance with the Patient Identification Policy.
- Arrange the necessary personnel for transferring the patient (e.g. a doctor, nurse, paramedic, healthcare assistant or porter).
- Ensure the admission and nursing assessment document, care plans and risk assessment booklet are fully completed and photocopied if necessary. Collect any additional healthcare records (or copies if the transfer is external to the Trust), including x-rays and any test results that are required for the transfer.
- Ensure that, where required, healthcare records are booked out of the patient's current inpatient setting via the Patient Administration System (PAS) Tracer System and in accordance with the Healthcare Records Policy.
- Ensure that, where an infection is suspected or identified, confirm whether isolation is required and alert the receiving unit, ward, department or hospital and SWAST, in accordance with the Patient Isolation and Staff Exclusion Policy and the Standard Infection Control Precautions Policy.
- Ensure that, where appropriate, the patient is adequately sedated or made comfortable with analgesia and or antiemetic prior to transfer.

- Ensure that the patient is adequately dressed or covered prior to transfer, and that the patient is warm and as comfortable as possible.
- Ensure patient is securely and safely positioned with appropriate harnessing (ICS 19.6.1)
- Rationalise all infusions, feeding and treatments that can be temporarily with held for the duration of transfer.
- Ensure sufficient infusions are drawn up and ready for administration during transfer i.e. inotropes, sedation, paralysing agent, fluids, blood etc
- Ensure that all necessary information about the patient's medicines is accurately recorded and transferred with the patient, and that responsibility for ongoing medicines management is clear. This includes any medicines that may need or are due to be administered during transfer.
- Ensure patient specific medicines are transferred with the patient to facilitate continued treatment on arrival at receiving hospital.
- Ensure the patient has been offered adequate food and drink prior to transfer or provide a snack box if the patient is likely to miss a regular meal.
- Ensure that all the necessary transportation equipment is present, in full working order, with batteries fully charged and that there is a sufficient supply of oxygen to last for transfer. If a patient is being transferred for a diagnostic test/ assessment, ensure the supplied oxygen is sufficient to last until their return in accordance with the Oxygen Policy.
- Ensure any required monitoring equipment or medical devices are attached and properly secured to allow access to administer medication and prevent equipment becoming a missile in the event of emergency deceleration.
- Ensure transfer bag is checked ready for use in the event of an emergency during transfer and any additional patient specific requirements.
- Ensure a Critical Care Transfer Form is available for completion during transfer. See Appendix B.
- Ensure that all of the patient's property is securely packed and labelled in hospital property bags or the patient's own luggage. Patients property Policy.

5.3 During Transfer

See Appendix A.

- All staff must practise standard infection control procedures during transfer for all patients, regardless of their infection status.
- Transfer personnel should be suitably trained in the use of all equipment and must ensure attention is given to all equipment, e.g. intravenous lines, tubes, drains or infusion devices remain secured throughout the transfer to prevent them from becoming dislodged or disconnected.
- Transfer personnel should carry out observations and document on the Critical Care Transfer form during the transfer. See Appendix B.

- On arrival at the destination, ensure that the patient is formally handed over to receiving staff and the top copy of the transfer form left in the patient's notes at recipient site. The patient should be transferred onto the receiving units equipment and infusion administration confirmed.
- All equipment used during transfer should be returned to the unit, cleaned appropriately and put on charge.
- Remaining transfer documentation should be processed appropriately according to instructions on form.

5.4 Repatriations

Guidance on the transfer of repatriated patients, either from another Trust to Northern Devon Healthcare NHS Trust, or from Northern Devon Healthcare NHS Trust to a home Trust, can be found in the Bed Management Policy.

6 Training requirements

All staff are required to undertake transfers of critically ill adult patients must be confident and competent in the care of critically patients and will have undergone individualised training by senior nursing staff on all transfer equipment. It is expected staff will gain experience in intra hospital transfers before undertaking an inter hospital transfer. Medical staff undertake a formal one day training course on critically ill transfers.

Due to location patients are often transferred by helicopter. Familiarisation flights have been organised and accessed by medical and nursing staff with the RAF Search and Rescue Team based at RMB Chivenor. Debriefs are conducted following individual transfers to establish what went well and what we could change. Staff are encouraged to write up transfers for personal portfolio's and evidence for annual Development and Reviews.

7 Monitoring Compliance With and the Effectiveness of the Policy

7.1 Standards/ Key Performance Indicators

Key performance indicators comprise:

- Routine monitoring of incidents.
- Routine monitoring of complaints and patient feedback.
- Network critical care transfer audit undertaken by Torbay hospital.

7.2 Process for Monitoring Compliance and Effectiveness

Monitoring compliance with this policy will be the responsibility of the Senior Nurse/Matron Critical Care. This will be undertaken by

- Monitoring of incident report forms, and the identification of individual incidents, trends and themes will be discussed and / or action planning at the Critical Care Staff meeting and at the Patient Safety Operational Committee.
- Weekly checks of all transfer equipment will take place.
- Annual reports of the Torbay Critical Care Transfer Audit will be requested and disseminated to the Critical Care Team.

Where non-compliance is identified, support and advice will be provided to improve practice.

Monitoring Arrangements

Compliance of this policy against all minimum requirements in the NHSLA Risk Management Standards will be monitored on a continuous basis with a continuous assessment of complaints and incidents.

Responsibility

Clinical Lead and Senior Nurse Critical Care will be responsible for monitoring and reporting to the Patient Safety Operational Group.

Reporting Arrangements

The Patient Safety Operational Group will develop an action plan to improve compliance and ensure improvements in performance occur, following review of incident trends and themes.
Action plans will be implemented by the Head of Quality and Safety to ensure learning takes place.

The Patient Safety Operational Group will monitor progress of the action plan on a monthly basis and exceptions will be reported via this group to the Safer Care Delivery Committee. Identified risks related to the non-compliance with this policy through audit will be registered on the Trust Risk Register system by the Risk Co-ordinator.

8 References

Intensive Care Society Standards and Guidelines. Guidelines for the transport of the critically adult (3rd Edition 2011) located at: http://criticalcaremedicine.pbworks.com/f/Transport+of+Critically+Ill+Patient~ICS.PDF

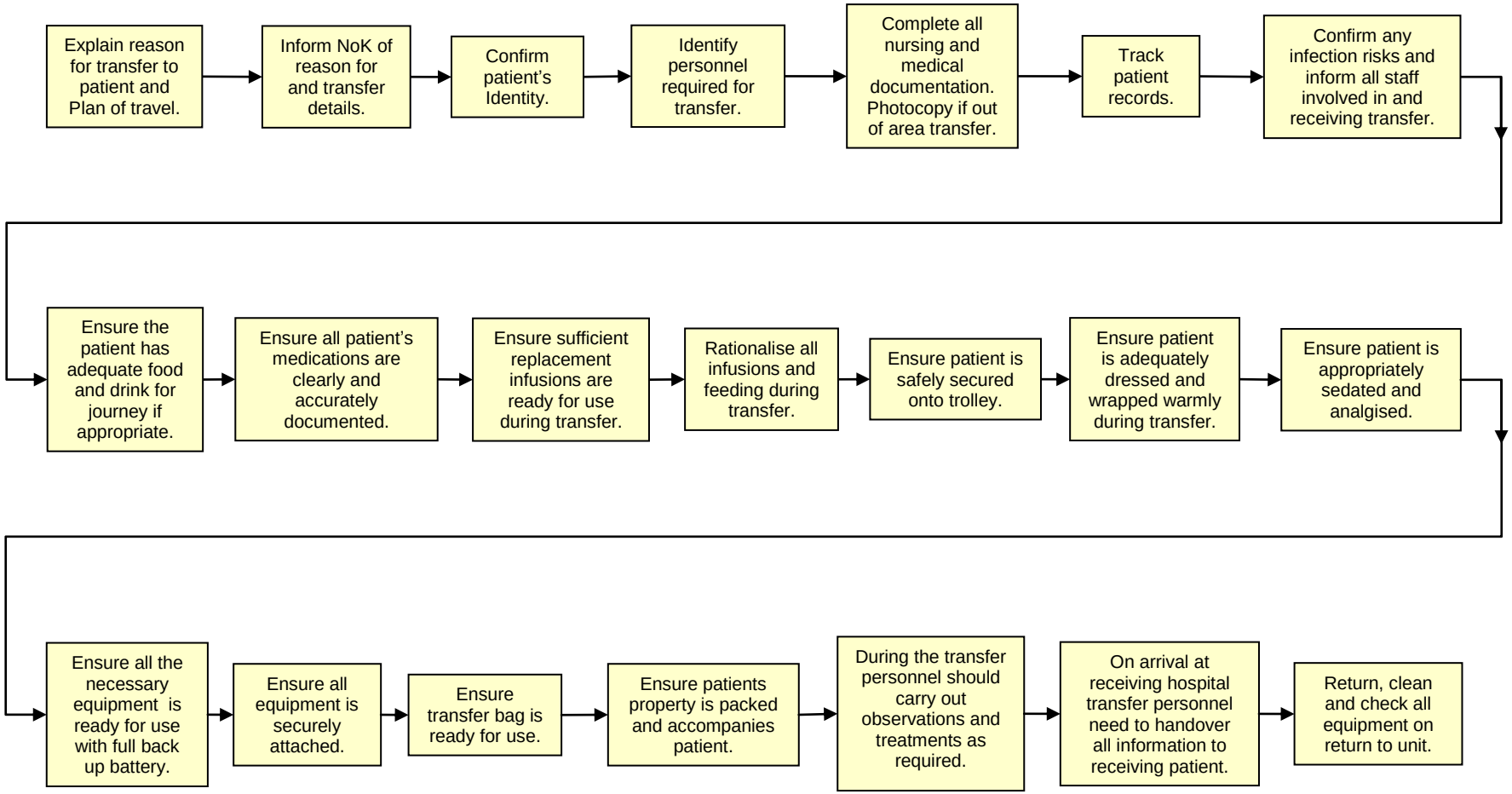
- Comprehensive Critical Care: A review of adult critical care services. Department of Health London.U.K. 2000 http://www.dh.gov.uk/en/Publicationsandstatistics/Publications/PublicationsPolicyAndGuidance/DH_4006585
- Torbay Audit reports available from Critical care Secretary, Department of Critical care Medicine, Level 5 Torbay Hospital, Lawes Bridge, Torquay, TQ2 7AA.

9 Associated Documentation

- Bed Management Policy
- Discharge Policy
- Healthcare Records Policy
- Management of Patient at Risk Policy
- Medicines Policy
- Moving and Handling Policy
- Oxygen Policy

- Patient Identification Policy
- Patient Isolation and Staff Exclusion Policy
- Patient Property Policy
- Resuscitation Policy
- Risk Management Policy
- Standard Infection Control Precautions Policy
- Transfer and Escort of Adult Patients Policy

Appendix A: Transfer flowchart



Appendix B: Critical Care Transfer Form

South West

Critical Care Transfer Form

To be used for all patients transferred to Critical Care - this is a legal record of transfer

PATIENT DETAILS

Name .....

Address.....

.....

..... Post Code.....

Age or DOB..... Male ☐ Female ☐

ESCORTING PERSONNEL

Doctor

Name .....

Grade ..... Spec.....

Transfer training ☐ YES ☐ NO

Nurse/ODP

Name .....

Band..... Spec.....

Transfer training ☐ YES ☐ NO

TRANSFER DETAILS

Date of Admission to Hospital .....

Date of Transfer .....

Transferred from: ICU ☐ WARD ☐

.....Hospital A&E ☐ THEATRE ☐

OTHER ☐

REASON FOR TRANSFER

To.....Hospital

Repatriation ☐ No staffed bed space in CCU ☐ No bed space in CCU ☐

SPECIALIST TREATMENT

Yes ☐ (please state) ☐ Other (please state) ☐

Pre-Sedation GCS E /4 V /5 M /6

DIAGNOSIS

1)

2)

3)

Checklist - Must be completed before transfer

Decision made by consultant ☐

Checklist 1 & 2 completed (see reverse of form) ☐

Approx O_2 requirements ☐ .....Litres

Patient notes photocopied ☐

Results/X-rays/Scans present ☐

Relatives aware of transfer ☐

Name of person completing checklist (please print)

.....

CONTACT DETAILS

Time receiving hospital contacted ☐☐

Name of contact and bleep .....

.....

Time patient ready for transfer ☐☐

Time receiving hospital accepted patient ☐

Reasons for any delays.....

.....

AMBULANCE DETAILS

LAND / AIR AMBULANCE / MILITARY HELICOPTER *Please circle*

Time contacted ☐ Ambulance Reference Number ☐

Time left unit ☐ Time arrived at receiving ICU ☐☐

VENTILATION DURING TRANSFER

Please tick appropriate boxes: Spontaneous ☐ Mechanical ☐

ET Tube Size / Length at Teeth ☐

Ventilator Type ☐

Start

Tidal Volume (VT) ☐

Peak Inflation Pressure ☐

PEEP ☐

FiO_2 ☐

RR ☐

BLOOD TESTS

Blood Glucose ☐

ABG ☐ $ETCO_2$ ☐ FiO_2 ☐

Other ☐

OBSERVATION CHART for use during transfer

TIME	HRS	Pre Transfer																							
use 24 hr clock	MINS																								
Drugs																									
Pupils size/reaction L																									
R																									
Rhythm																									
SpO_2																									
$ETCO_2$																									
Respiratory rate																									
FLUIDS	220																								220
	200																								200
1	180																								180
2	160																								160
3	140																								140
	120																								120
	100																								100
	80																								80
	60																								60
	50																								50
	40																								40
	30																								30
Please record MAP — SBP V DBP Λ	Fluids																								Fluids
	Urine																								Urine
	Temp																								Temp

LINES SITE ☐ Arterial ☐ Central ☐ Peripheral ☐ Vas Cath

....

Other please detail.....

SPINE Precautions taken for spinal injury

☐ n/a ☐ Yes Details.....

SUMMARY OF TRANSFER including comments/problems

Dr's name *(please print)*.....

Signature of Escorting Doctor .....

CRITICAL INCIDENTS

Cardiac arrest		Other
Profound arrhythmia		
Hypertension		
Hypotension		
Accidental extubation		
Intubation		
Convulsions		
Dislodged cannula		
Monitor failure		
Ventilator failure		
Pump failure		
Oxygen failure		
Pupil dilatation		

COMMENTS OF RECEIVING DOCTOR

Signature of Receiving Doctor..... Dr's name *(please print)*.....

INSTRUCTIONS

Form must be taken with patient and completed by escort staff once patient is transferred.

WHITE	Insert in Patient Notes at Recipient Site
YELLOW	Post to Critical Care Unit Secretary, Torbay Hospital or using attached envelope
GREEN	Fix into the Transferring Hospital's Notes